

RECOVERY AND RECURRENCE

When clients are released from an inpatient treatment center, or they are working on their changing their addictive patterns with the help of outpatient therapy, they are fragile and emotionally vulnerable. They face interpersonal and work challenges without their drug or practice of choice. They generally feel alone, are grieving past losses, and still crave relief. This is a moment when another firefighter might step in to pinch-hit—for example, by bingeing and purging instead of drinking—or when the client might just return to their preferred addictive process. The protective system activates in response to exiled vulnerability coming back into consciousness. This is known as a *recurrence* (we prefer the term to relapse, which is shaming), and it causes the psyche to ignite with familiar suffering. One over-excited part ricochets off the next in a scrum of self-disgust, self-loathing, fury, and fear (*Maybe I can't get better!*).

Traditional treatment often questions the client's intentions (*Do you even want to get better?*), blames them for recurring symptoms (*How many meetings have you attended? Have you reached out to your sponsor lately?*), and employs strong directives (*Just resist that craving!*) in the belief that this will help the client regain control. While a sense of imperative about helping the client get back on track is understandable, this approach is unlikely to be helpful. If we weigh in at this point from the perspective of a worried, advice-giving manager, we are making the mistake of pressing a thumb on the manager side of the scale. So, in IFS, we do something different. Rather than helping the client get back in control, we guide them to stay in close contact with their recently deposed firefighters and get back into relationship.

We remain positive, compassionate, and most of all, calm. Our job is to convey confidence that we can get to the bottom of how this started and what keeps it going. Our assurance is essential. We listen, knowing the client will feel better when their Self reconnects with their protectors and has access to their exiles. We assure clients that new Self-to-part patterns are empowering and they're not back at square one. We remind them that all this is very predictable. Recurrence is similar to backlash and is a normal aspect of the healing process. We remind the client that cutting back or being sober could release a surge of painful memories, older and more recent. We caution that critics might respond with shaming and that, in response, firefighters would step up with the usual temptations. We remind the client that the addictive process is a cycle with its own rationale and its own aims. It does make sense.

Now that the cycle is back in action, we can name the parts and ask them to slow down. We don't work on improving protector behavior; we work on improving the client's ability to reach out to protectors, understand their motives, and build attachment. We help them see that their compulsive parts, who believe the client is a vulnerable child who needs relief, are making choices. In this way, we move clients from confusion to clarity. With their new perspective on using, clients can see that their firefighter parts aren't just crazy and destructive, they are opting to survive. Recurrence is thus another opportunity to calm protectors and proceed with the project of connecting with exiles. This approach is affirming and optimistic. We find that most clients can be curious about why they are using again. They can also make use of new practices to rebalance emotionally.

Therapist Handout

An Intervention Option after Recurrence

This exercise is intended to help the client step back from fear and self-condemnation and understand why their parts are once again using a substance or engaging in a compulsive practice in a problematic way. Here we address a blended system and ask protective parts to step back and trust us. A sample script is provided for you throughout, as well as some guiding prompts to walk you through each step of the intervention.

1. Remind the client that the addictive cycle makes sense and that by inquiring together, you can get to the bottom of the most recent recurrence.

When protective parts get worried, they fall back on the old tactics they trust. Firefighter parts might begin drinking, gambling, bingeing, using drugs, or engaging in any other compulsive behavior that aims to soothe or distract. In turn, manager parts might step in with shaming or expressions of disgust. This is normal and predictable. When this happens, we can ask firefighter parts about their intentions. They generally say they're worried that exiles will overwhelm the system with painful emotions, old memories, or untold secrets. In this exercise, we will invite them to explain their reasons. If a compulsive behavior alarmed any other parts, we can help those parts too.

2. When the connection between the client and therapist is firm, guide the client to invite their protective parts to a meeting.

Let's have a summit meeting. Let's invite all the parts who need to be here right now to join us. Invite the compulsive firefighter parts and the critical manager parts to come sit at a big table with you. They are all welcome. Who needs your attention first?

3. Hear from whatever part needs the client's attention first. For example, a critical manager might share about their fear of chaos and emotional overwhelm. A stirred-up firefighter might discuss their fears of exile overwhelm and managerial shaming. Either way, reassure the part that you are here to listen.

We are not here to control or criticize you. We want to understand your intentions. What are you concerned about? How were you trying to help?

- If a firefighter is reacting to a manager:

If we could help that manager part stop controlling or caretaking or overworking, would you need to _____ (use substances, gamble, overeat, watch pornography, etc.) in the same way?

- If a manager part is reacting to a firefighter:

If we could help that using part so it would _____ (keep its agreements, find a more benign way to distract, etc.), would you need to keep attacking it?

- If either protective part is afraid of an exile:

If you saw there was a safe way for that injured part to feel wanted and loved, would you be willing to downshift a bit?

4. Ask both protector teams to let the client's Self help the exile they protect (it may be the same exile or different exiles). When they agree, welcome the exile by beginning with a one-way Self-to-part bid for connection.

Let that hurt young part know that you are here, that you are paying attention. Let it know that it's not alone. How does it respond?

5. If the Self and the exile connect and protectors are still supportive, continue with a two-way connection and witness the exile.

What does this part want you to know? What caused its hurt, loneliness, or sense of shame? Are you taking this in? Let it know. Can it feel your concern and care? How does it respond?

6. When this feels complete, clarify the cycle of disconnection by inviting the client to draw and discuss their addiction triangle. Which protectors react to each other? Which exiled part (s) get triggered? Does this feel familiar or new? Then draw and discuss another triangle that depicts what happens when the client's Self is in the lead.

Do the parts feel connected to you now? What do they need from you to keep toning down? Thank them for taking the risk of being with you today. Do they want homework or a set plan to work on before next session?

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* IFS borrows from the shamanistic tradition of unburdening to the elements to help exiles complete the process of leaving the past and letting go of limiting, painful beliefs about who they are. Michi Rose, an early collaborator in the development of IFS, introduced this feature of unburdening.

* Thanks to Michi Rose for the golden thread exercise.